

Leveraging Programmatic Assessment for Certification Decisions: Lessons From Pediatrics, Family Medicine, and Surgery

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Place Assessment Platform

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ABMS Annual Conference

September 2026



BETTER CARE
BETTER FUTURES

FACULTY
DEVELOPMENT

LEARNER
GROWTH

FEEDBACK &
COACHING

DATA &
IMPROVEMENT

ASSESSMENT
DESIGN

SYSTEMS &
POLICY

Learning Objectives and Outline

By the end of this session, each participant will be able to:

- Describe three specialties' approaches to implementing CBME within assessment and certification processes
- Compare key design decisions and trade-offs — workplace-based assessment, defining practice readiness, and alignment with certification
- Identify transferable implementation lessons — and pitfalls — for their own boards and programs

OUTLINE

15 min Surgery

Jacob A. Greenberg, MD, EdM

15 min Pediatrics

Ariel Winn, MD

15 min Family Medicine

Warren Newton, MD, MPH

15 min Discussion + Audience Q&A

David A. Turner, MD



THE AMERICAN
BOARD OF SURGERY

FROM EPAs TO CERTIFICATION

Turning Frontline Observations into Defensible Judgments of Readiness

Jacob A. Greenberg, MD, EdM
AMERICAN BOARD OF SURGERY
ABMS Conference 2026

Disclosures

- Jacob A. Greenberg, MD, EdM – Associate Professor of Surgery with Tenure, Duke University
 - J and J MedTech
 - Consultant
 - Research Support
 - Medtronic
 - Consultant
 - Research Support
 - American Board of Surgery
 - Councilor

The Assessment Problem

Certification asks a simple question. Our traditional data often do not.

Is this trainee ready to perform the work of a surgeon independently?

Case logs

What was done

Rotation evaluations

How someone was remembered

Milestones

A periodic synthesis

EPAs add evidence from the work itself.

The ABS Strategy

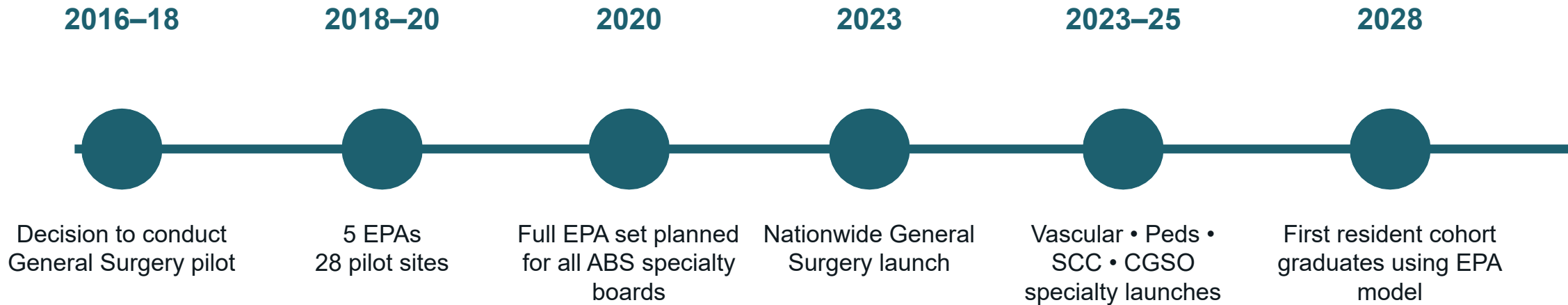
Move assessment closer to the clinical encounter—and aggregate the signal over time.



One observation is not the answer. The pattern is.

ABS EPA Timeline

From pilot to national implementation—and toward the first graduating EPA cohort.



A deliberately long on-ramp preceded national scale.

One Framework with Multiple Assessment Platforms

Programs can capture frontline EPA assessments through different technology solutions.

SIMPL

Mobile workplace-based
assessment

FIREFLY

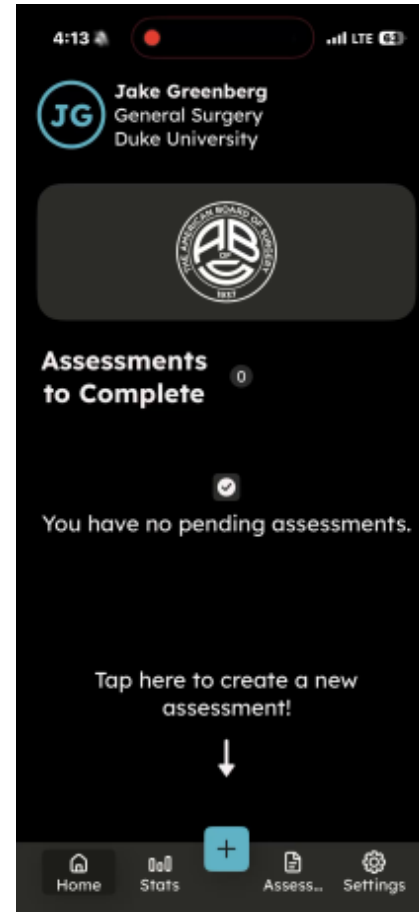
EPA assessment platform

SEPA

EPA assessment platform

The platform is the interface. The assessment framework—and the evidence it generates—is the point.

How Hard is it to Complete an Assessment?



Easy Enough To Do Once..... Powerful at Scale!

114,306

2023–24

289,783

2024–25

360,765

2025–26 to date

General Surgery EPA assessments

~370 programs

approximate

~6,000 trainees

approximate

The Model is Expanding Across Surgery

EPA implementation is no longer limited to General Surgery.

Specialty	EPAs	Programs	Trainees	Start
Vascular	19,035	200	851	Aug 2025
Pediatric Surgery	5,433	52	102	Aug 2025
Surgical Critical Care	5,426	175	381	Sep 2025
CGSO	5,002	35	144	Aug 2025

Program and trainee counts are approximate.

But Volume is Not Validity

The challenge is turning many observations into a trustworthy signal.

SAMPLING

Enough observations
across contexts

QUALITY

Meaningful ratings +
narrative

TRAJECTORY

Progress over time, not
snapshots

SYNTHESIS

Human judgment
informed by data

More data help only when they improve the decision.

What is Being Assessed?

Assessment spans operative and nonoperative work, with broad coverage across the EPA set.

65%

Procedural / Intraoperative

35%

Nonoperative

>50,000

Gallbladder

20,000–49,999

Appendicitis • Abdominal wall hernia • Colon disease

15,000–19,999

Consult • Trauma • Inguinal hernia • Breast disease

10,000–14,999

Anorectal • Critically ill • Cutaneous neoplasms • Soft tissue infection • Endoscopy • Acute abdomen

5,000–9,999

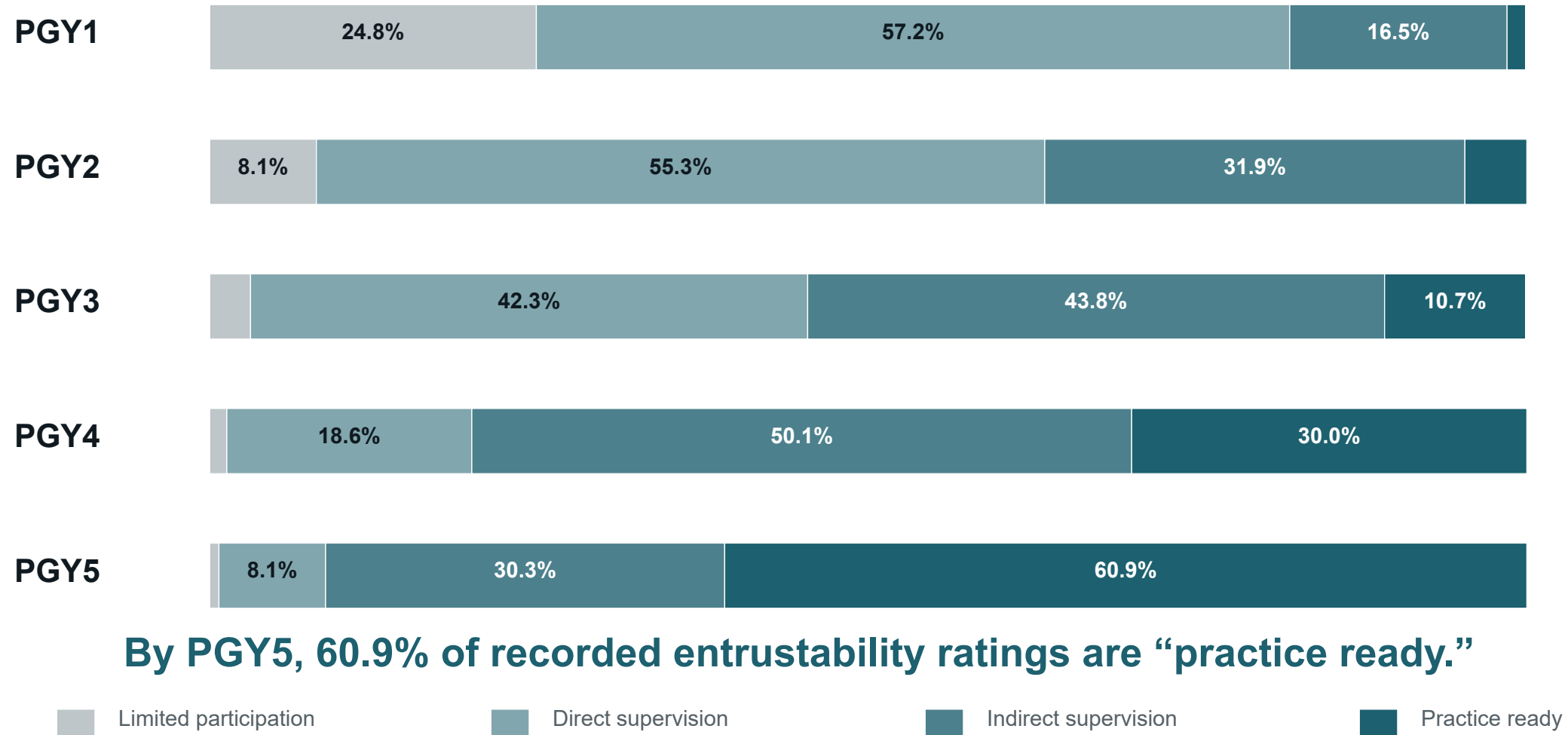
Renal replacement therapy • Thyroid/parathyroid • SBO

<1,000

Pancreatitis

Entrustability Progresses with Training

The distribution shifts from direct supervision toward practice readiness across PGY levels.



What Changes For The Clinical Competency Committee?

Programmatic assessment should change the conversation, not just add another dashboard.

FROM

“How do we think this resident is doing?”

- episodic impressions
- recency effects
- variable exposure

TOWARD

“What does the accumulated evidence show?”

- repeated observations
- progression over time
- evidence tied to clinical work

EPA data inform judgment; they do not eliminate judgment.

The Next Frontier: Certification Decisions

National implementation establishes feasibility. Certification requires a higher evidentiary bar.

Can longitudinal workplace-based assessment become part of a fair, valid, defensible determination of readiness for independent practice?

What evidence is enough?

Sampling + representativeness

How should it be synthesized?

Rules, judgment, or both?

How do we ensure fairness?

Across programs, raters, and contexts

What belongs with the Board?

Program data → certification evidence

What Surgery Has Learned So Far

A national programmatic-assessment system is an implementation project as much as an assessment project.

1

Make the frontline task easy

If assessment is burdensome, sampling collapses.

2

Allow technology flexibility

One framework can live across multiple platforms.

3

Design for aggregation

The value is in patterns across time and observers.

4

Keep people in the loop

Data should strengthen—not replace—expert judgment.

The American Board of Surgery

THE GOAL IS NOT

360,000 assessments.

THE GOAL IS

a better certification decision.

Frontline observation → longitudinal evidence → trustworthy judgment



**THE AMERICAN
BOARD OF SURGERY**

Questions?

Jacob A. Greenberg, MD, EdM
AMERICAN BOARD OF SURGERY
ABMS Conference 2026

ABMS CONFERENCE · SEPTEMBER 2026

Rebuilding Certification Around Practice Readiness

The American Board of Pediatrics' road to 2028

Ariel Winn, MD

American Board of Pediatrics · Boston Children's Hospital / Harvard Medical School

Session moderated by David Turner, MD · with Jacob Greenberg, MD & Warren Newton, MD

THE AMERICAN BOARD *of* PEDIATRICS

Disclosures

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I receive funding from the American Board of Pediatrics for my role in this work.

COLLABORATION

The platform is developed in collaboration with the Society for Improving Medical Professional Learning (SIMPL).

THE DECISION

2028

Initial certification will be informed by program director attestation that each graduate can perform the General Pediatrics EPAs safely and effectively — without supervision.

The same story Dr. Greenberg told, in reverse — certification first, working back to the frontline.

Today's system wasn't built for this

Dr. Greenberg's map — and what pediatrics had at each step.



"Default decision-making is not defensible decision-making."

Graduation $\neq$ certification $\neq$ safe, effective care

79%

of program directors have graduated a resident despite concerns

53–98%

of residents deemed ready for unsupervised practice at graduation, by EPA

Nearly all

residents graduated and were board certified anyway

Contributing to this: the constructs and scales in use were inconsistent — and not always clear — likely leading to different interpretations.

Schumacher, West, Schwartz, et al., JAMA Network Open 2020 — 1,987 graduating residents · Schumacher, Kinnear, Poitevien, Daulton, Winn, Pediatrics 2025 — 117 program directors

What the attestation decision is driving

Start from what patients need — and work backwards.

1 Define the work

2 Define the bar

3 Generate the evidence

4 Support the judgment

} *the required outcome of training*

One question underneath: how do we ensure our trainees, once certified, can meet the needs of their patients upon entering practice?

Defining the work: 12 revised EPAs

The activities a general pediatrician performs in practice — and therefore must be able to do upon entering practice to meet their patients' needs.

- 1 Providing Preventative Primary Care for Children of All Ages
- 2 Providing Comprehensive Primary Care for Children With Complex, Chronic, or Special Health Care Needs
- 3 Managing Patients With Common Acute Diagnoses
- 4 Assessing and Managing Patients With Common Behavior and Mental Health Concerns
- 5 Caring for the Newborn Prior to Discharge
- 6 Recognizing a Patient Who Requires Subspecialty Medical or Surgical Care, Providing Initial Management, and Seeking Referral or Consultation
- 7 Recognizing a Severely Ill Patient, Providing Initial Management, and Mobilizing Resources Needed for Continued Care
- 8 Executing Clinical Handovers Within or Across Settings
- 9 Performing the Common Procedures of the General Pediatrician
- 10 Leading Interprofessional Teams to Provide Collaborative, Family-Centered Care
- 11 Promoting Equitable Care at the Level of Each Individual Patient and the Population to Address Racism and Other Contributors to Health Inequities
- 12 Utilizing Technology and Information Sources to Enhance Learning and Patient Care

O'Connor, Chiel, Lewis, Winn, et al., Pediatrics 2026;157(5)

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The bar is practice ready

Able to provide safe and effective care as a General Pediatrician — without an assigned supervisor, in a typical general pediatrics practice, with appropriate onboarding and the level of collaboration with colleagues usual in such a practice.

One standard — all 12 General Pediatrics EPAs, every tool and process aligned to it.

Winn, O'Connor, Aylor, et al., Pediatrics 2026;157(6)

What practice ready means — and doesn't

Safe and effective, not expert

the bar is foundational capability — not mastery

Unsupervised, not unsupported

no assigned oversight — collaboration and consultation are expected and encouraged

The discipline, not the next job

fellowship plans don't change the standard

A foundation, not a ceiling

many graduates advance well beyond it

*In a typical practice, with appropriate onboarding —
across all 12 General Pediatrics EPAs.*

Practice-ready for all twelve EPAs?

Yes.

What the attestation decision is driving

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2 Define the bar

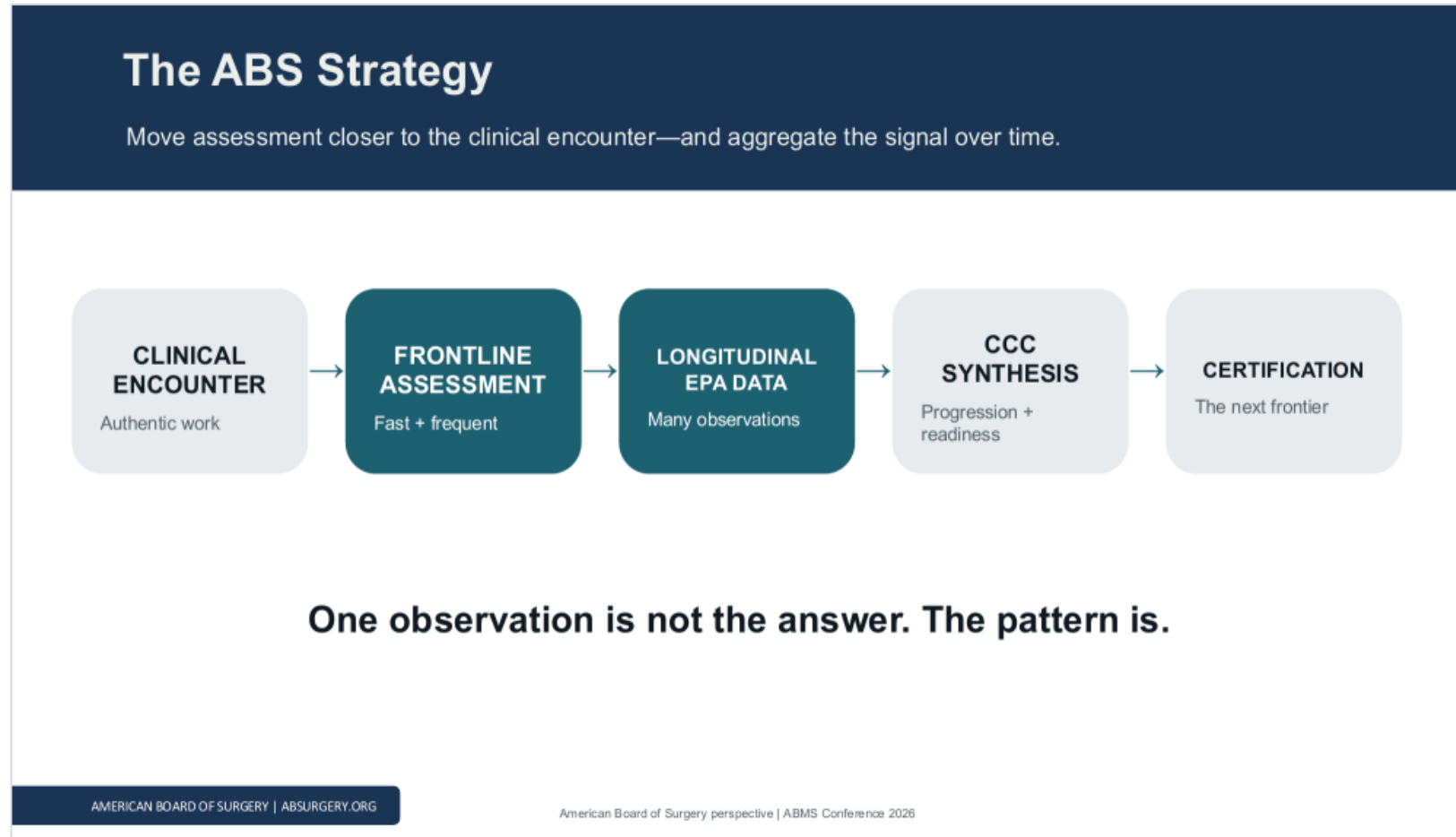
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The playbook is the same



Same playbook, different terrain

- **Less procedural**
- **Supervision is layered**
- **Training $\neq$ practice environment**

*Not a clean divide — as Dr. Greenberg showed,
a third of surgery's assessments are nonoperative, either.*

Four questions, every time

Q 1

Which presentation matches this patient?

narrows to the specific patient

Q 3

Supervision

a 1–5 scale, from “supervisor performed” to “resident-led, no supervision needed”

Q 2

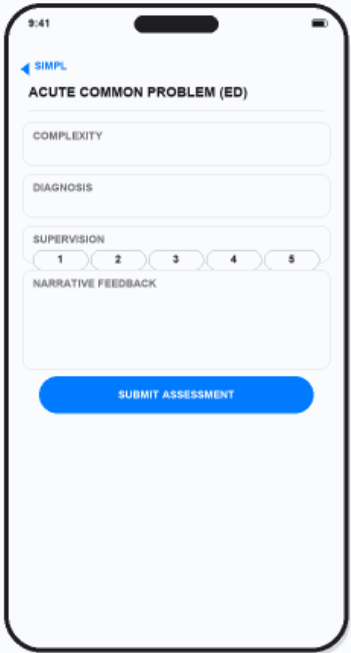
Complexity

straightforward · moderate · complex

Q 4

Narrative feedback

what did they do well? what would you suggest for next time?

A smartphone screen displaying a medical assessment form titled "ACUTE COMMON PROBLEM (ED)". The form includes sections for "COMPLEXITY", "DIAGNOSIS", "SUPERVISION" (with a 1-5 scale), and "NARRATIVE FEEDBACK". A blue "SUBMIT ASSESSMENT" button is at the bottom.

*acute common problem (ED) form —
animates in Slide Show*

Designed for under 3 minutes — 62% finish in ≤ 3 (median 2.4).

One construct, three altitudes

FRONTLINE

one encounter · retrospective

“How much supervision did the resident need when performing this activity?”

CCC

one EPA · prospective

“Could this trainee, practicing today without supervision, provide safe and effective care for this activity?”

PD ATTESTATION

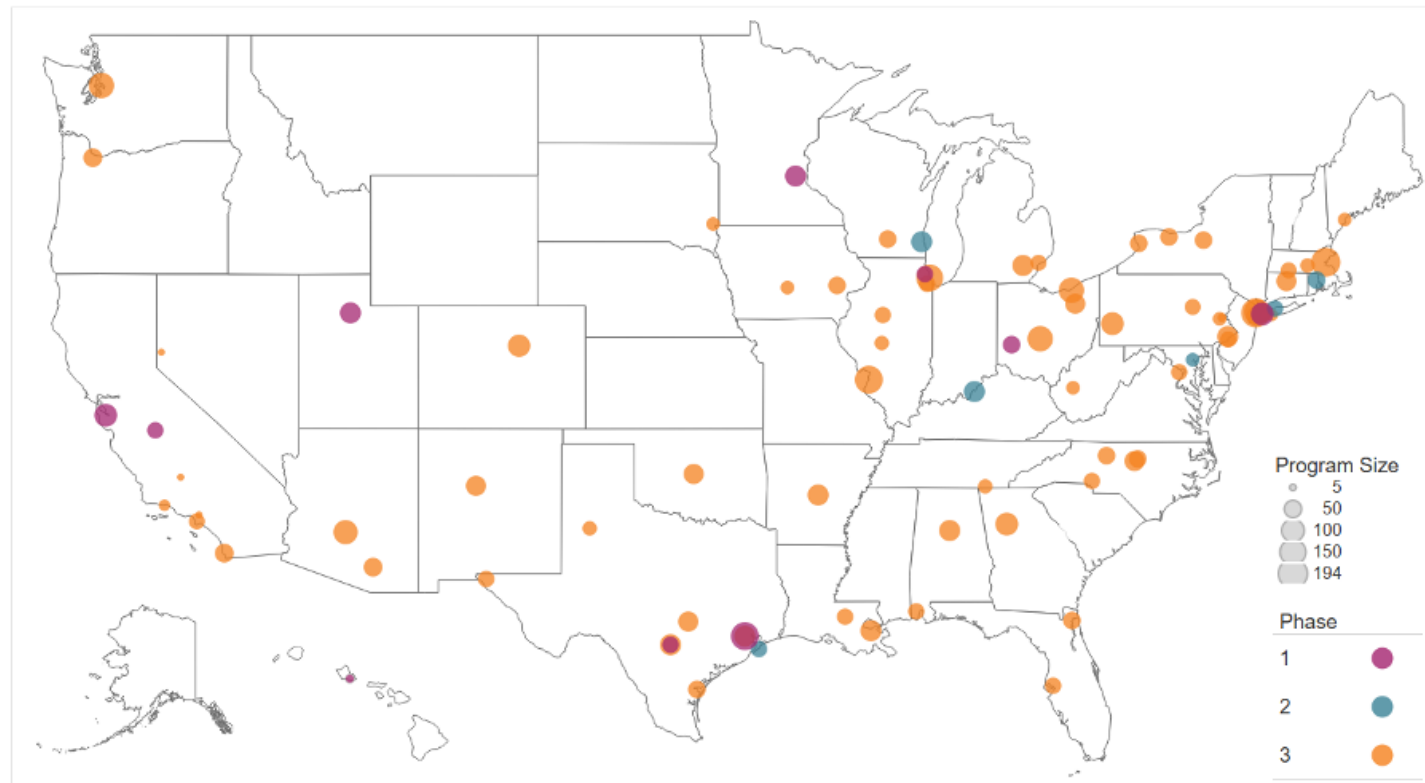
all 12 EPAs · at graduation

“Are they practice ready across all EPAs of the discipline?”

*The top of every scale is the same construct:
safe and effective care — without supervision needed.*

The evidence engine is running

Program Launch by Phase of Entry into SIMPL



each dot = one program · size = program size · color = launch phase

74

PROGRAMS

6,112

RESIDENTS

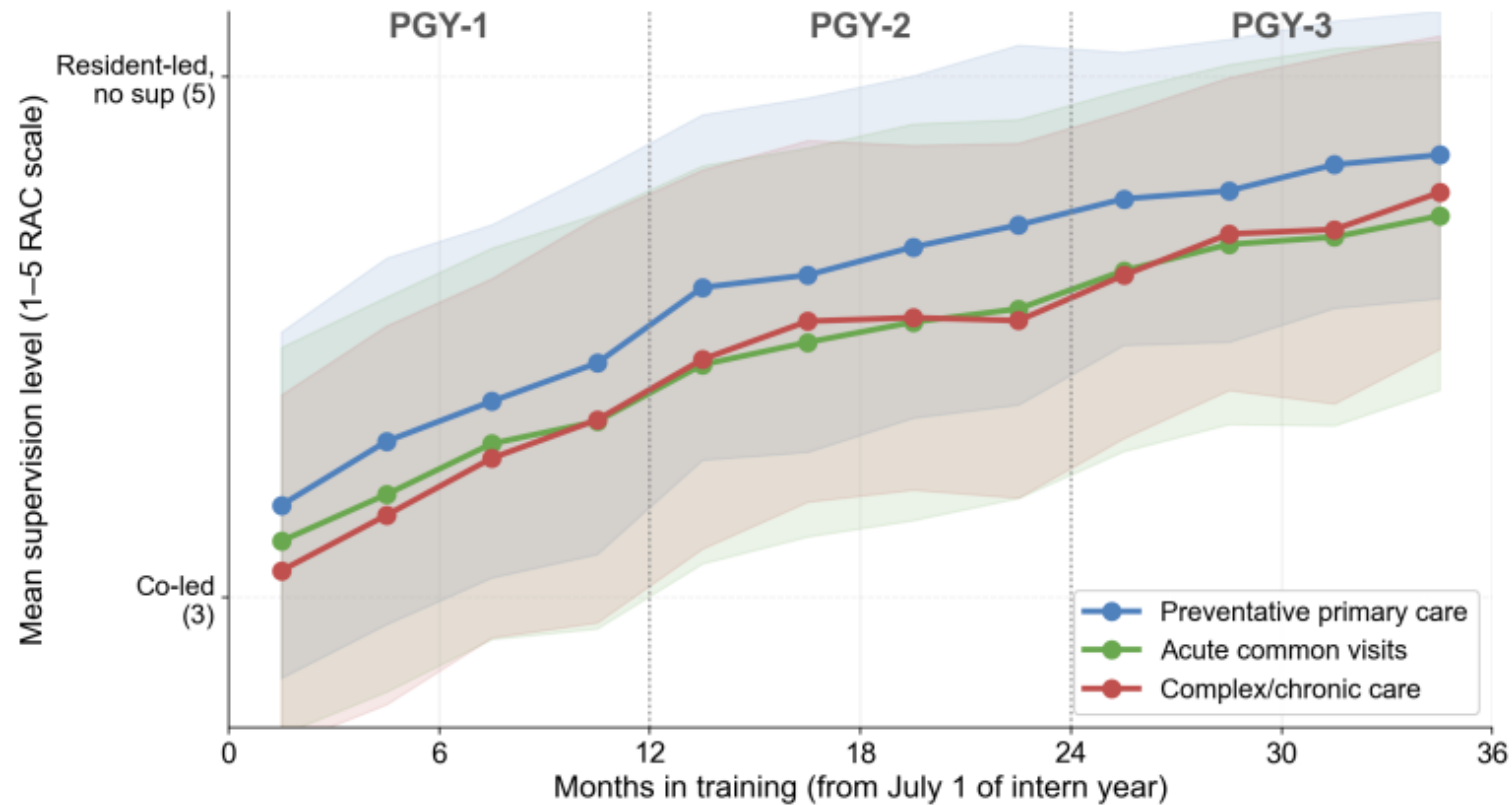
5,997

RATERS

103,878

ASSESSMENTS

Readiness becomes visible over time



3 EPAs pooled · quarterly bins · shaded ± 1 SD — April 2026 data, refresh before session

The same shape Dr. Greenberg showed in surgery

Three lessons

DESIGN

Measure the right construct imperfectly,
not the wrong construct precisely.

IMPLEMENTATION

Flexibility on HOW, standardization on WHAT.

BOTH

A tool is just a tool — culture change is the real work.

What the attestation decision is driving

Start from what patients need — and work backwards.

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2 Define the bar

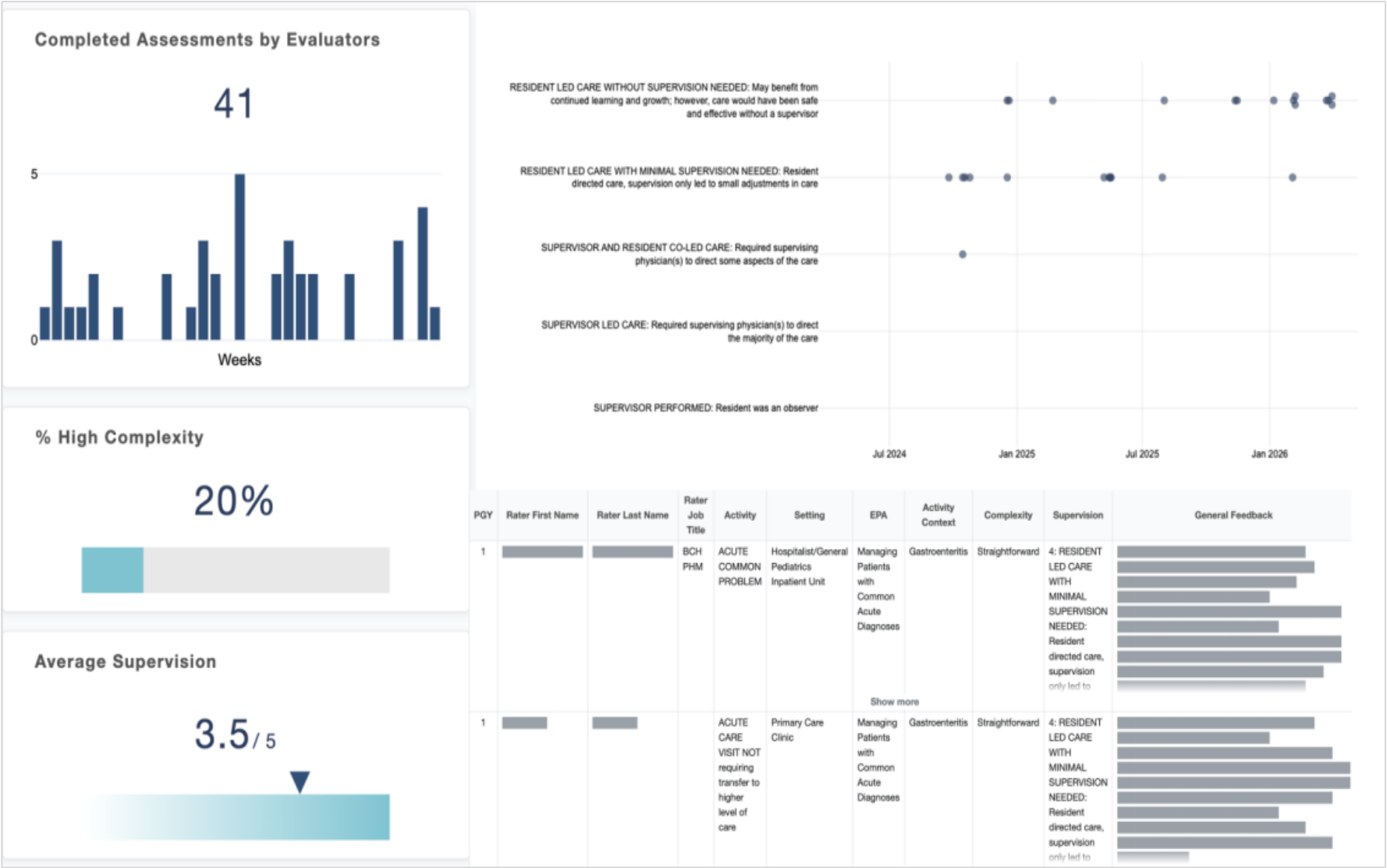
3 Generate the evidence

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The dashboards



From frontline data to defensible attestation

Frontline data → Dashboards by EPA → CCC judgment → **PD attestation**

AI-enabled narrative review

drawing signal from thousands of comments at scale

Strengthened CCC processes

tools and guides for the committees making the judgments

Cross-EPA translation

how evidence in one activity informs the rest

Toward 2028

Practice ready is the goal. Workplace-based assessment is a big leap toward it — one data stream among several that can support defensible decisions.

Scale & support

the growing network, lessons applied deliberately

More data streams

peers, patients, EHR data, and more

Research the translation

from data to defensible, equitable decisions

Validity

when is the evidence enough to trust

2028

The construct is defined.

The evidence engine is running.

The judgment is maturing.

Ariel Winn, MD · American Board of Pediatrics



The American Board Of Family Medicine

Residency Redesign & CBME in Family Medicine

Warren P. Newton, MD, MPH

President & CEO, ABFM

September 17, 2026

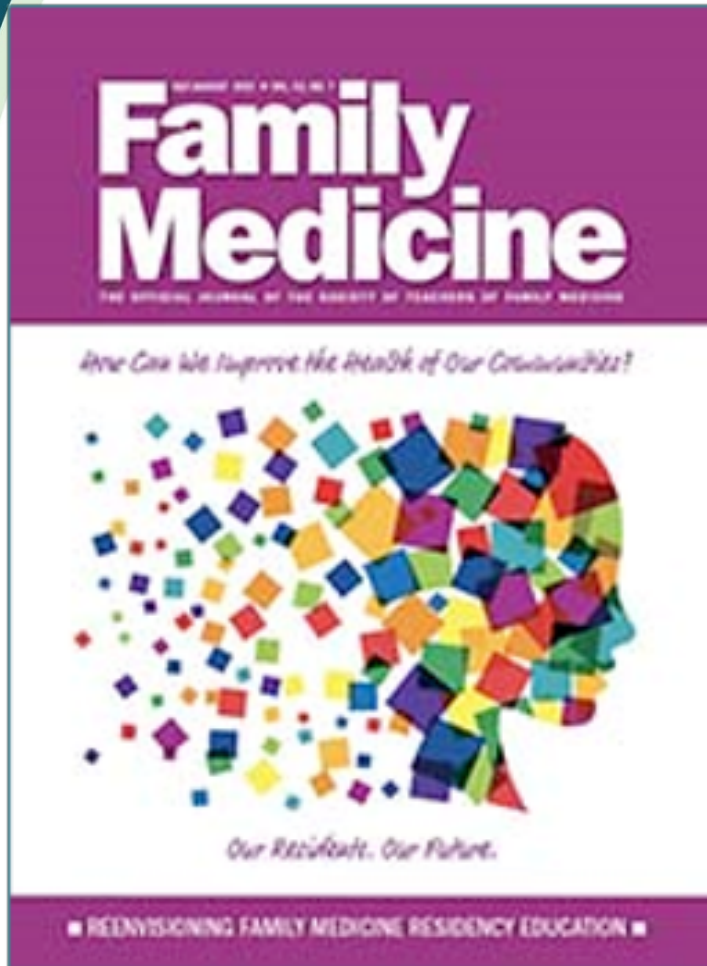
Disclosures

I am an employee of ABFM.

I have no other disclosures.

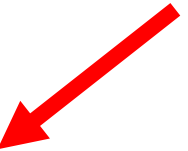


Family Medicine Residency Redesign



Key changes/innovations:

- Practice is the Curriculum
- Address Community Disparities
- ILPs and flexibility for residents and residencies
- Competency Based Assessment
- Residency Learning Networks
- Dedicated educational time



All organizations involved...

RC guidelines active 2023, Educational time restored 2024



Competency Based Board Eligibility: The Roadmap

15 Core Outcomes defined by
ABFM and ACGME Review
Committee--published March
2023, just before program
directors meeting

Active July 2024, along with new
RC guidelines

ABFM Requirements...

- Based on Core Outcomes
- Announced/published **June 2023**
- PDs/CCCs responsible, online verification
- Implemented **gradually 2024-26**
- Extension for **Core Procedures and Training** to 2027
- **POCUS** competence added for 2029



The Challenge

- **824 residencies, with 474 new residencies in 11 years; only 1/3 from single accreditation**
- **Increasing faster than other specialties; ~100 more in wings...**
- **~75% community programs – many in rural and frontier settings or without other residencies in sponsoring institution or substantial faculty development resources**

Faculty Development

Train the Trainer: collaboration with ACGME (n~80)

STFM CBME task force, with webinars and website: [Competency-Based Medical Education Toolkit for Residency Programs | Society of Teachers of Family Medicine](#)

Strengthening Outcomes and Assessment in Residencies (SOAR)—using 3 year outcomes to identify exemplars, annual pre-conference, website and a fellowship

Residency Learning Networks (n=31, over 30% of residents enrolled); both clinical and educational focus

FM AIRE: Extension of residency with additional competency (N=12 so far)

All funded by the ABFM Foundation



American Board
of Family Medicine

Using Data to Shape Residency Redesign

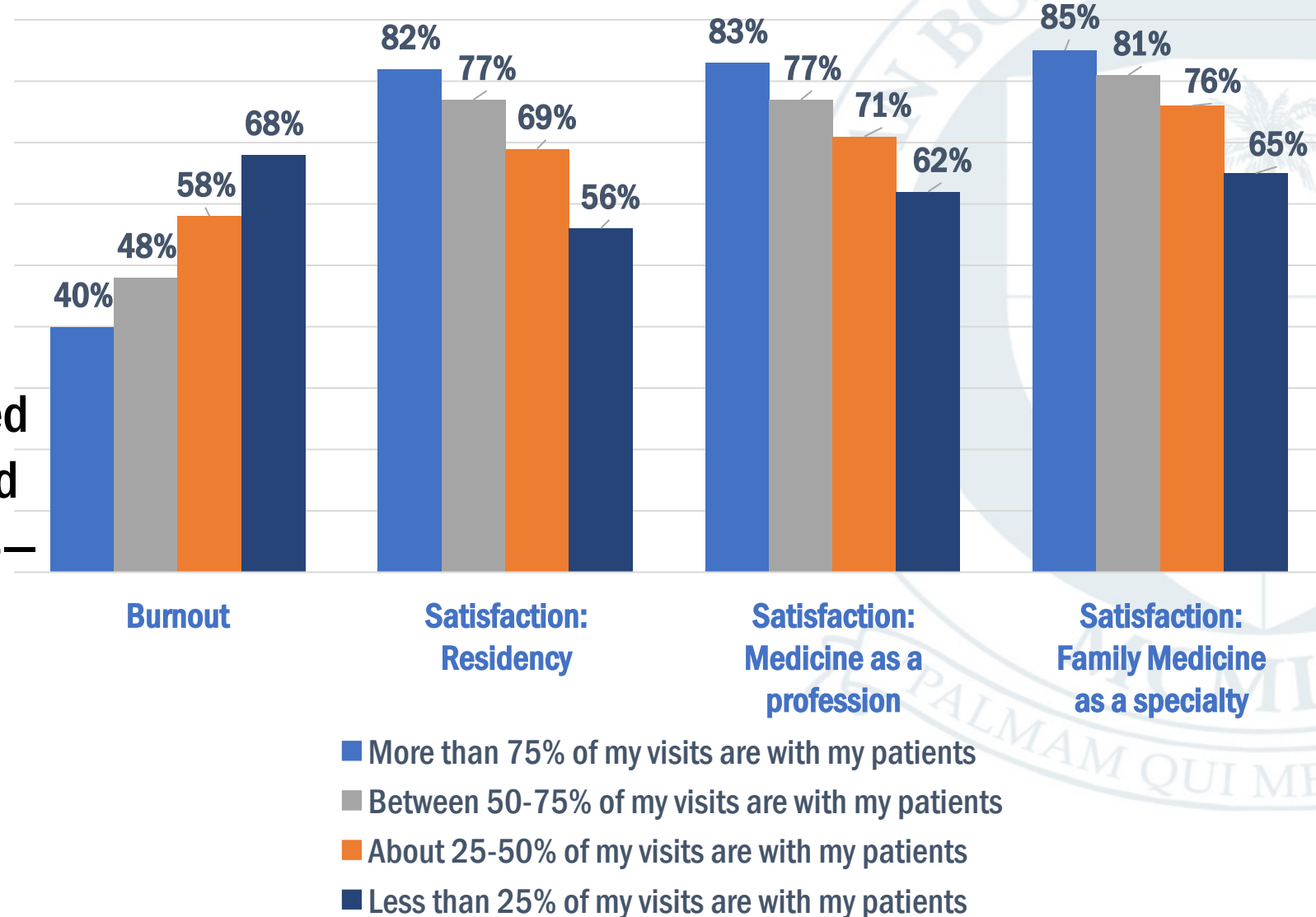
- **Residents' Voice:** 2025 National Resident survey—99.6% response rate, **15,341** residents
- **Verification Survey** (**almost all** program directors at end of residency)
- **In-Training Examination Trends** (**almost all residents**)
- **Graduate and Diplomate Surveys**—**3 years** after graduation and every recertification
- **ACGME** data complementary and increasingly available...
- **Rapid feedback to community at spring meetings and publications 3-6 months later**



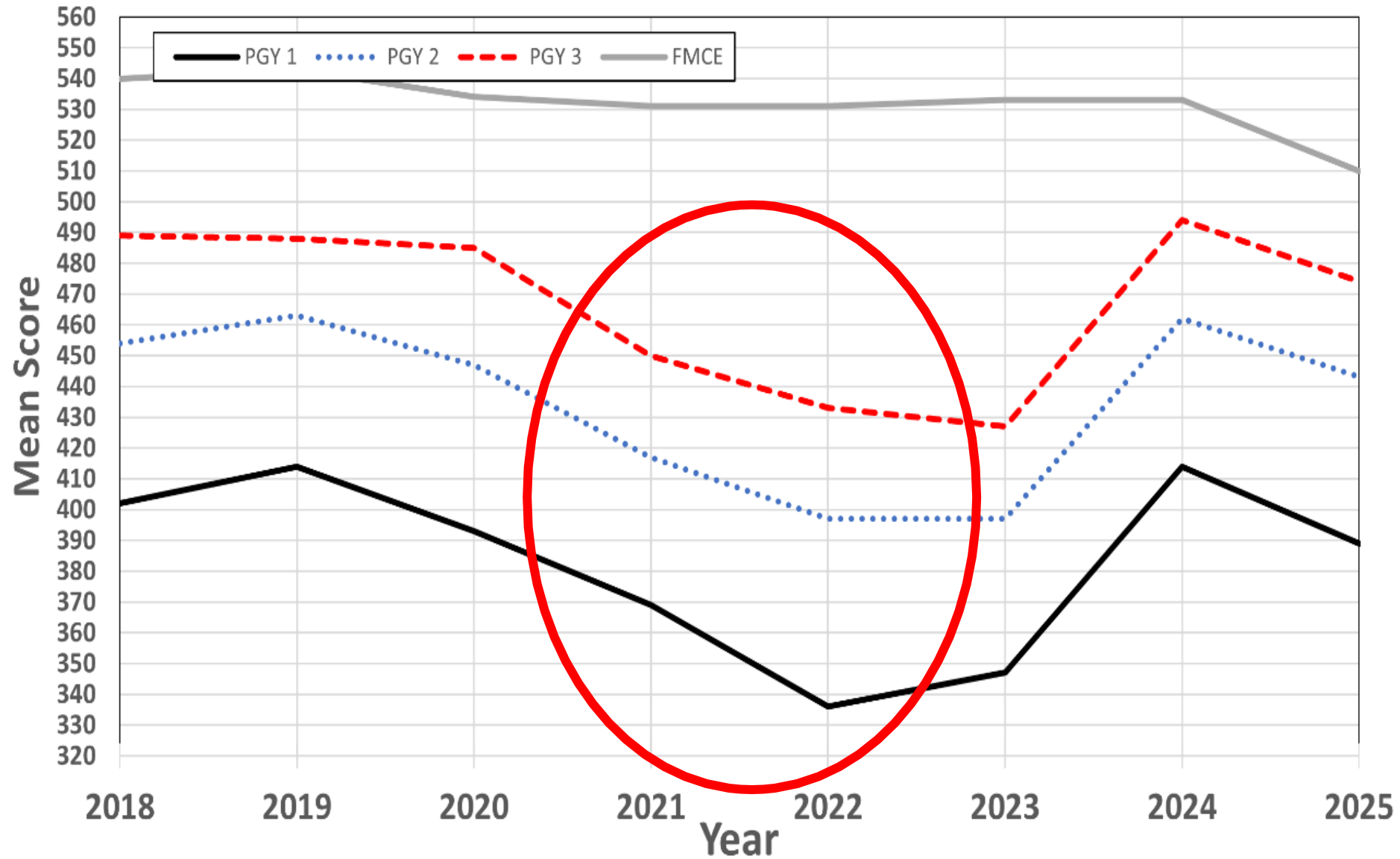
The Clinic is the Curriculum: Empanelment and Continuity...

In 2023, the Family RC required empanelment and 40% continuity for PGY3s

By 2024, 92% of residents reported empanelment, continuity increased to ~.45 in 2024 and >.50 in 2025—for all residents in the US!

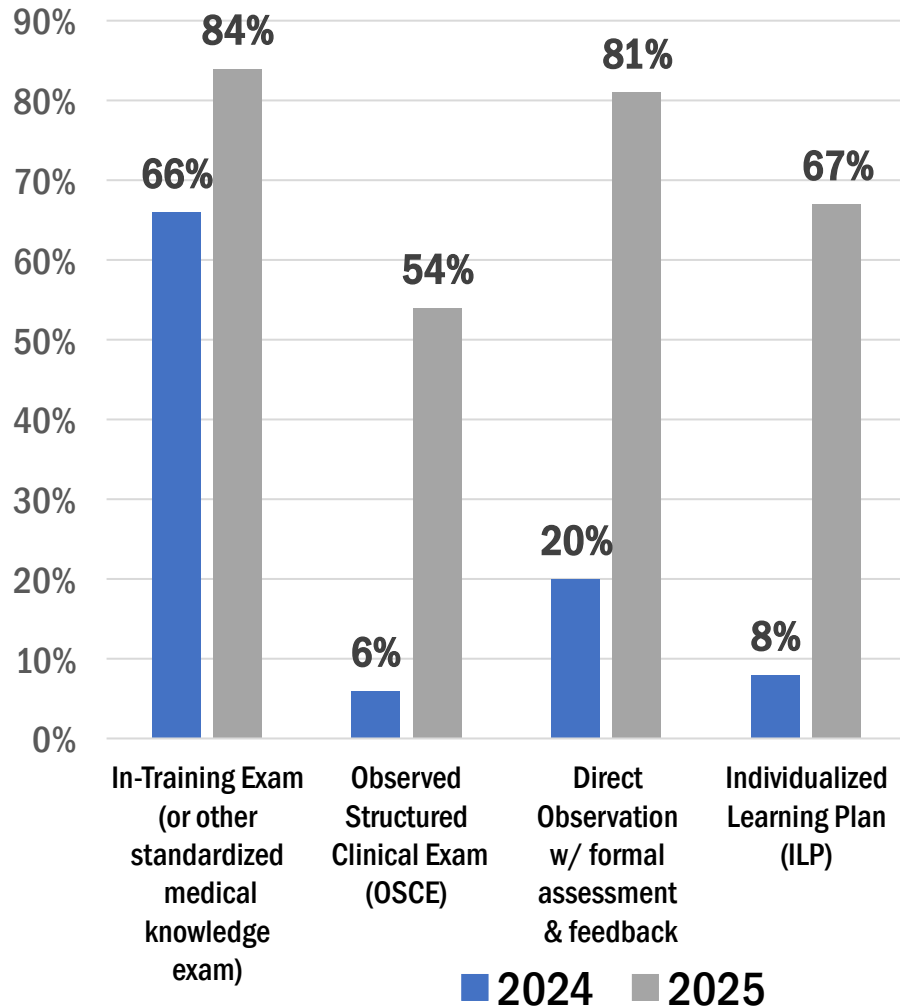


Mean ITE Scores

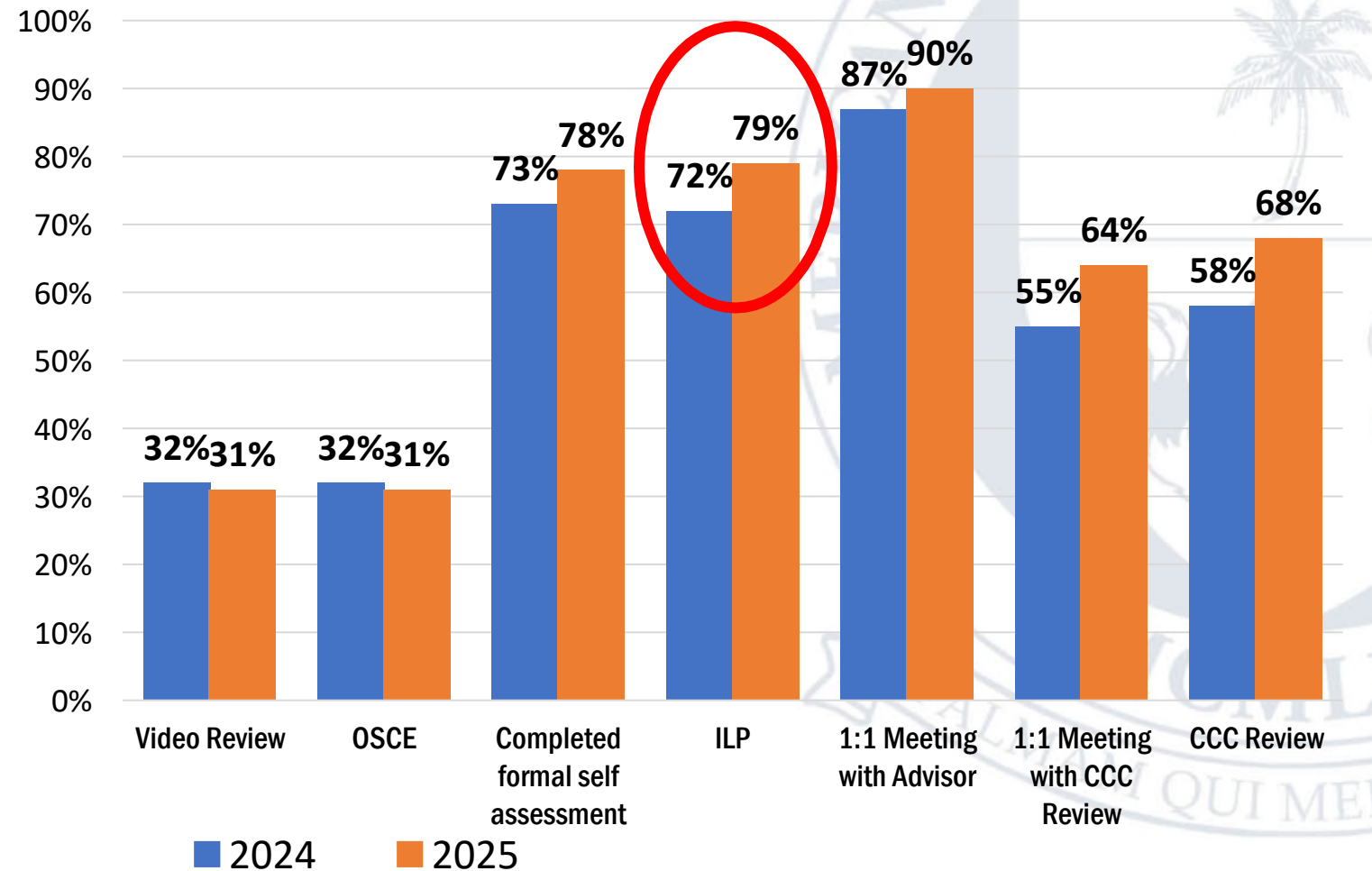


CBME Process: Onboarding, ongoing...

Orientation

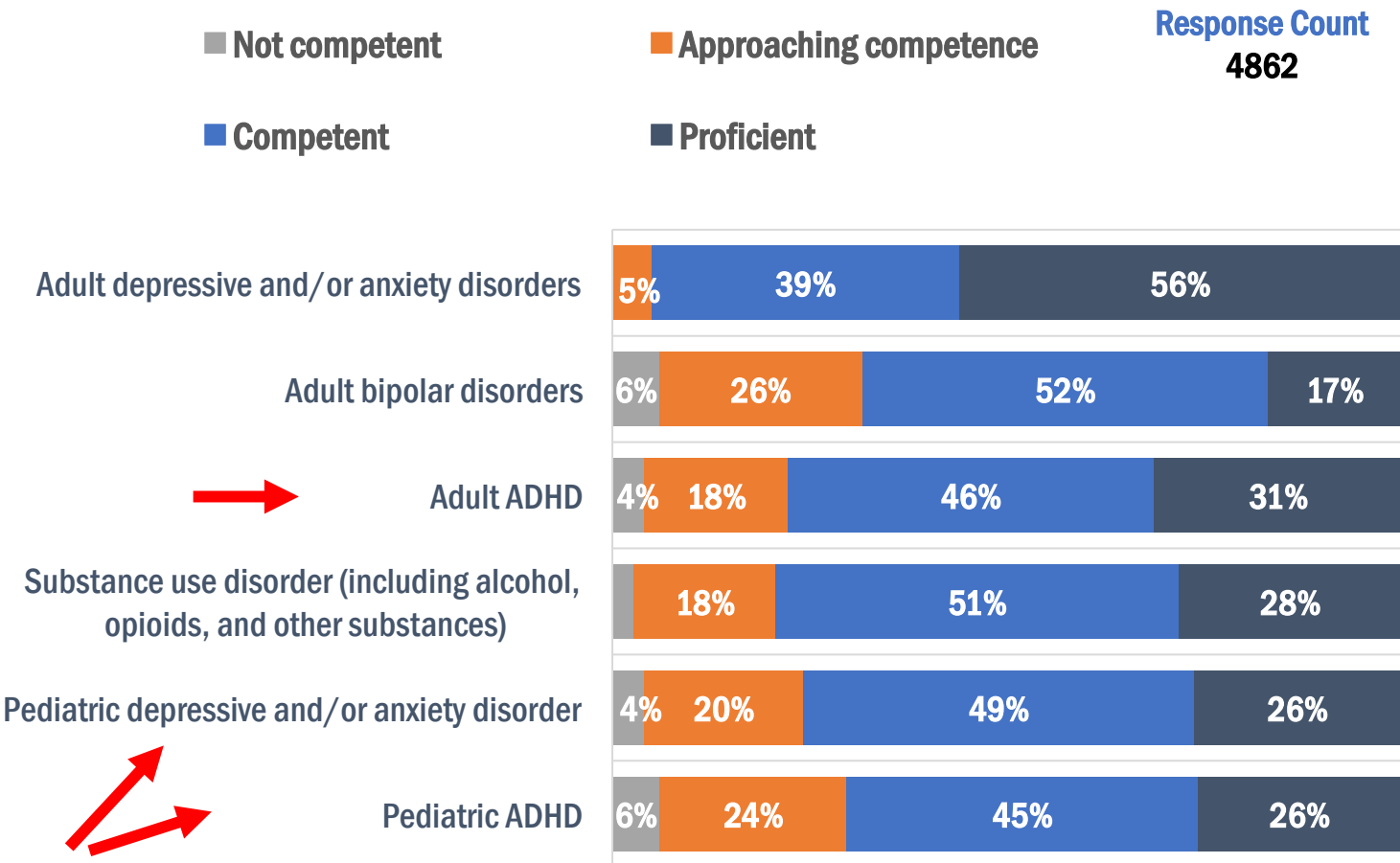


Last six months...

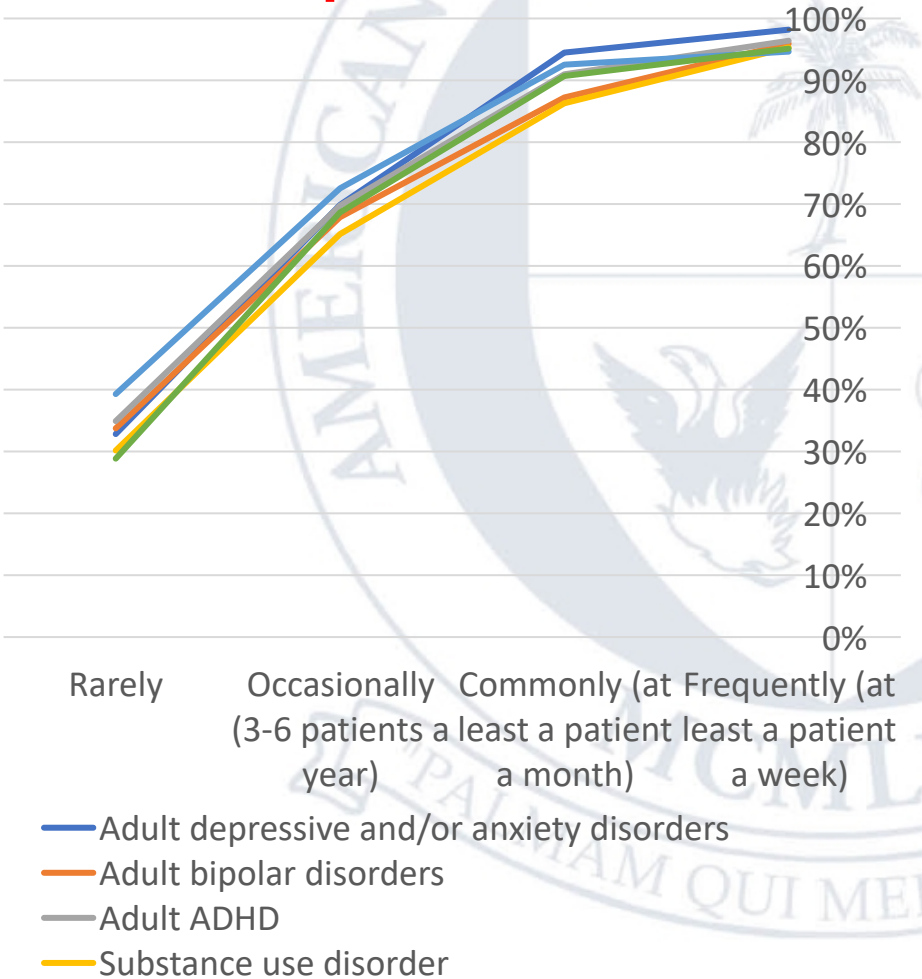


Mental Health

How would you rate your ability to **diagnose and manage common mental health problems** at completion of residency?



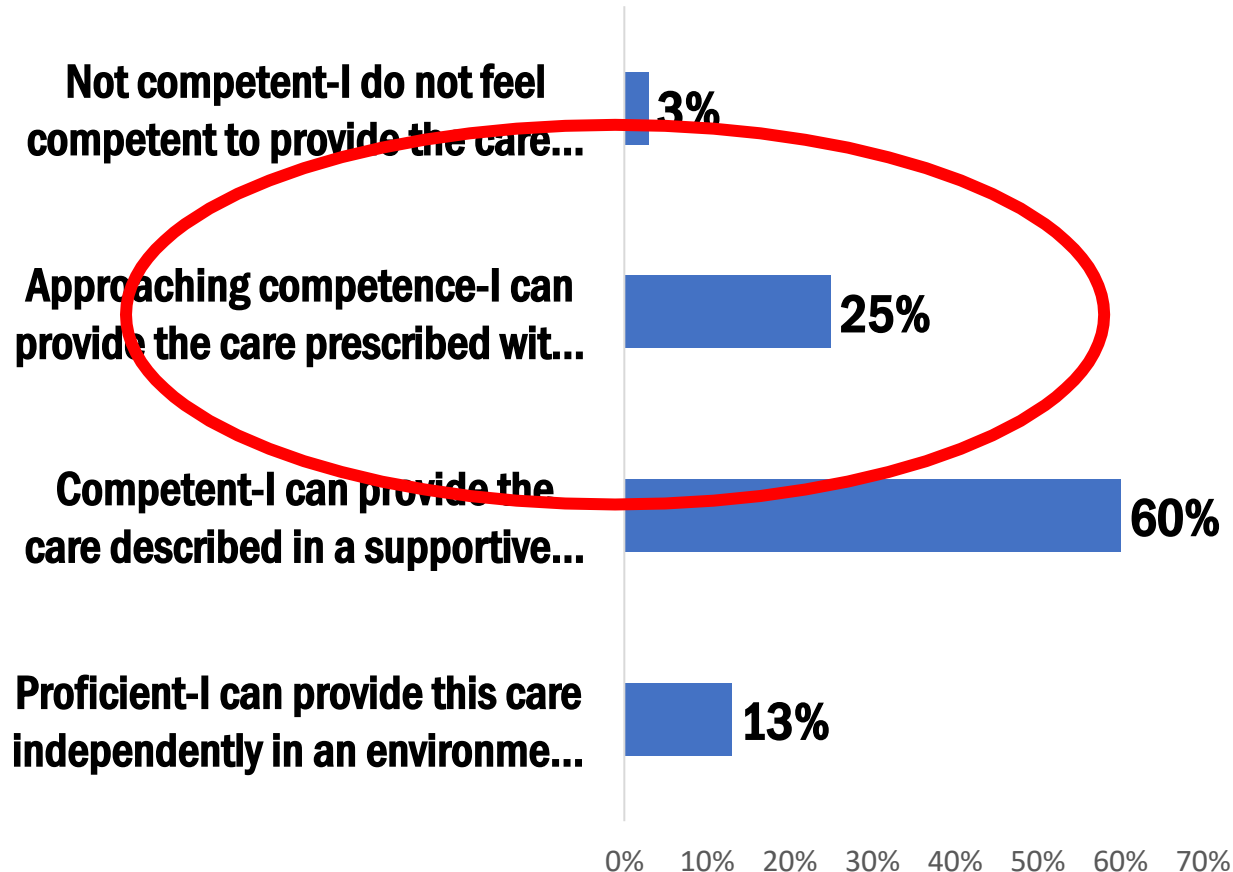
Impact of Continuity Exposure on Competence



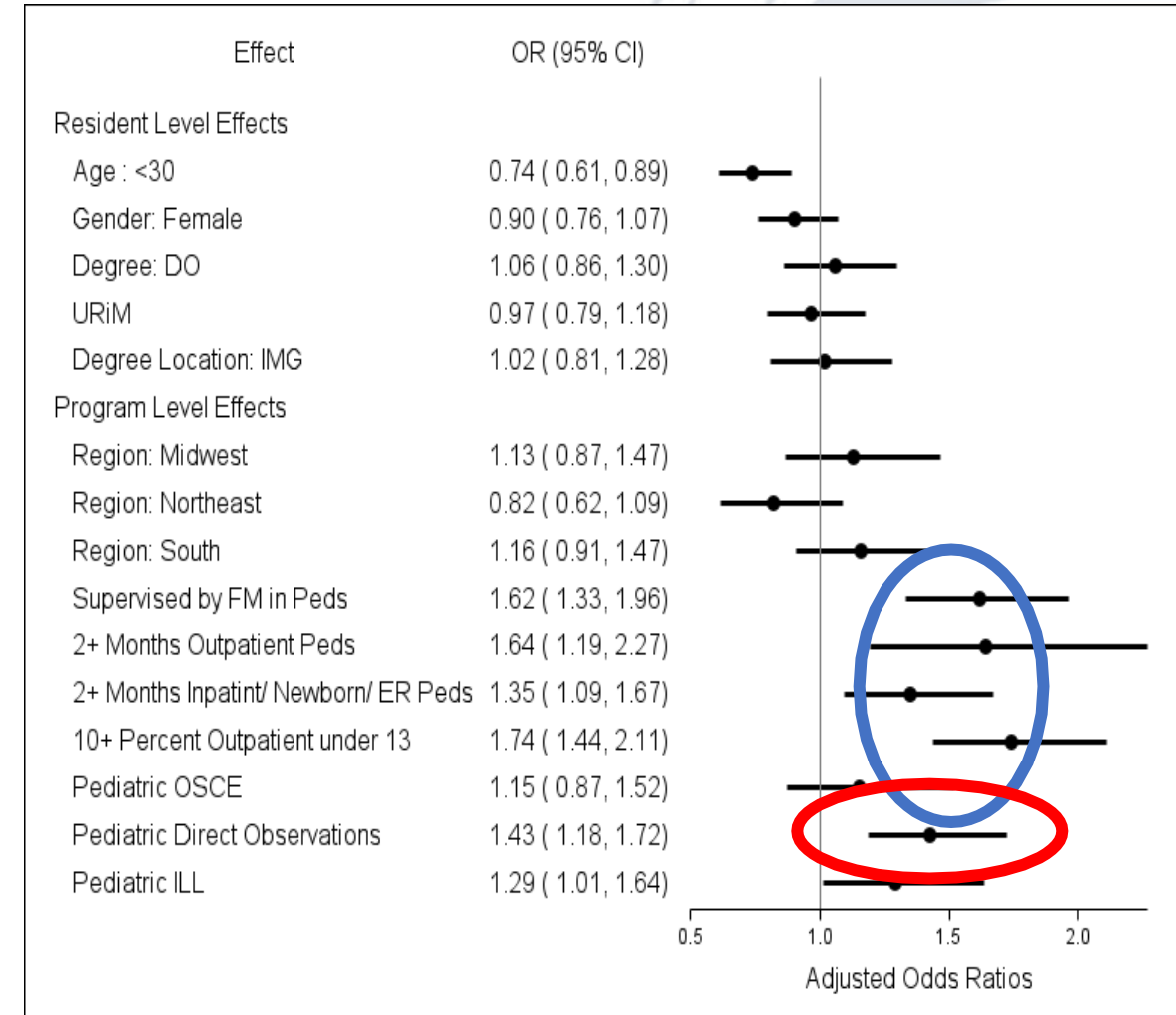
CBME Focus: Care of Children

**How would you rate your ability
to care for children
independently in practice?**

October
PGY3
N= 4788



Influences on Self Assessed Competence





Assessments: How many and what kinds?

Median #/resident: 50-75 (*no change but*
increased skew: >175/resident: 11% to 16%

Areas for improvement

- **Increasing volume and precision:** do precepting *role definition* or *incentives* work? Should CCCs *target* kinds of observations?
- Promote coding/billing, referral analysis...

Assessment	2023	2025
Direct Observation	95%	97%
Procedures	86%	91%
Chart Review	72%	78%
Self Assessments	87%	89%
Multisource	68%	68%
Coding/Billing	48%	57%
Referral Analysis	8%	16%



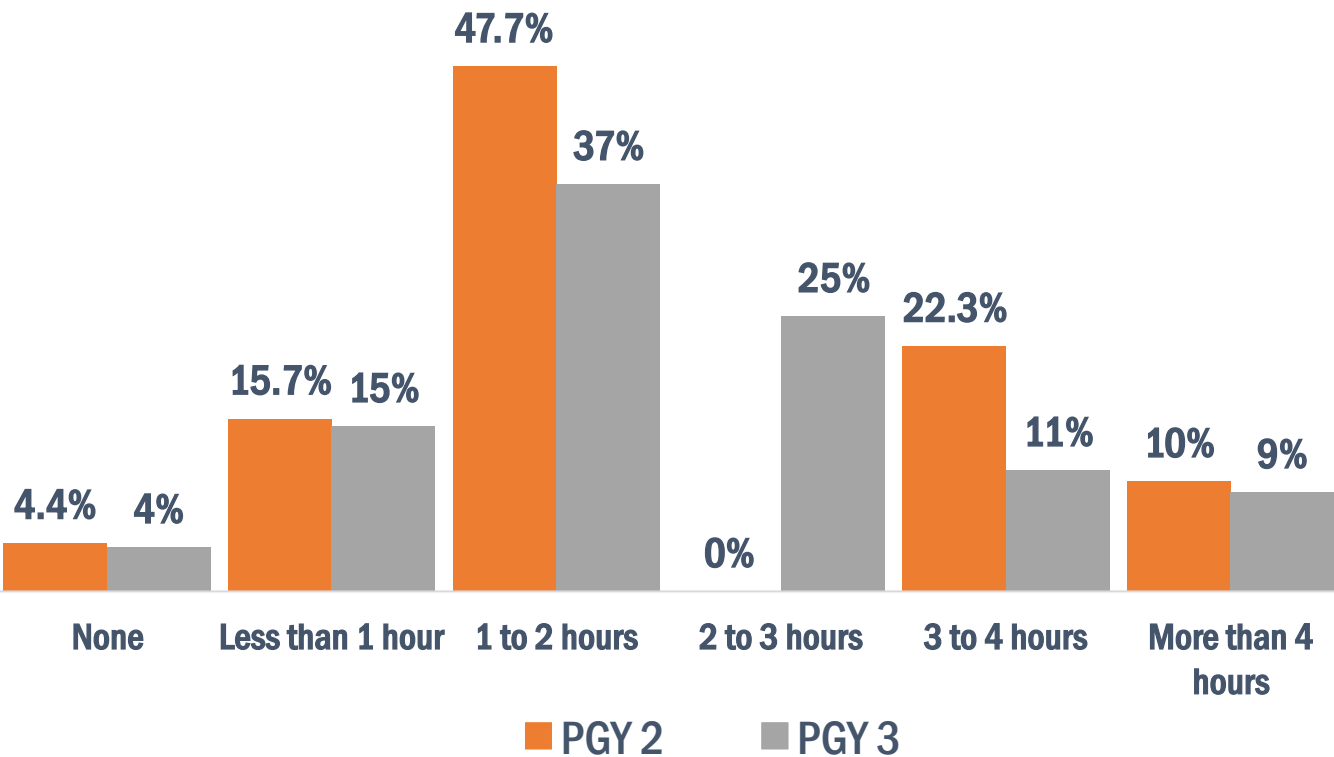
Outcomes: ABFM Competency Based Board Eligibility

- Online verification worked smoothly...
- Residents not verified--
 - 2024: 4/4867
 - 2025: 6/5003
 - 2026: 3/4850 so far
- **Too few vs too many?**
- *2/3rds professionalism!*
- Remediation ad hoc...



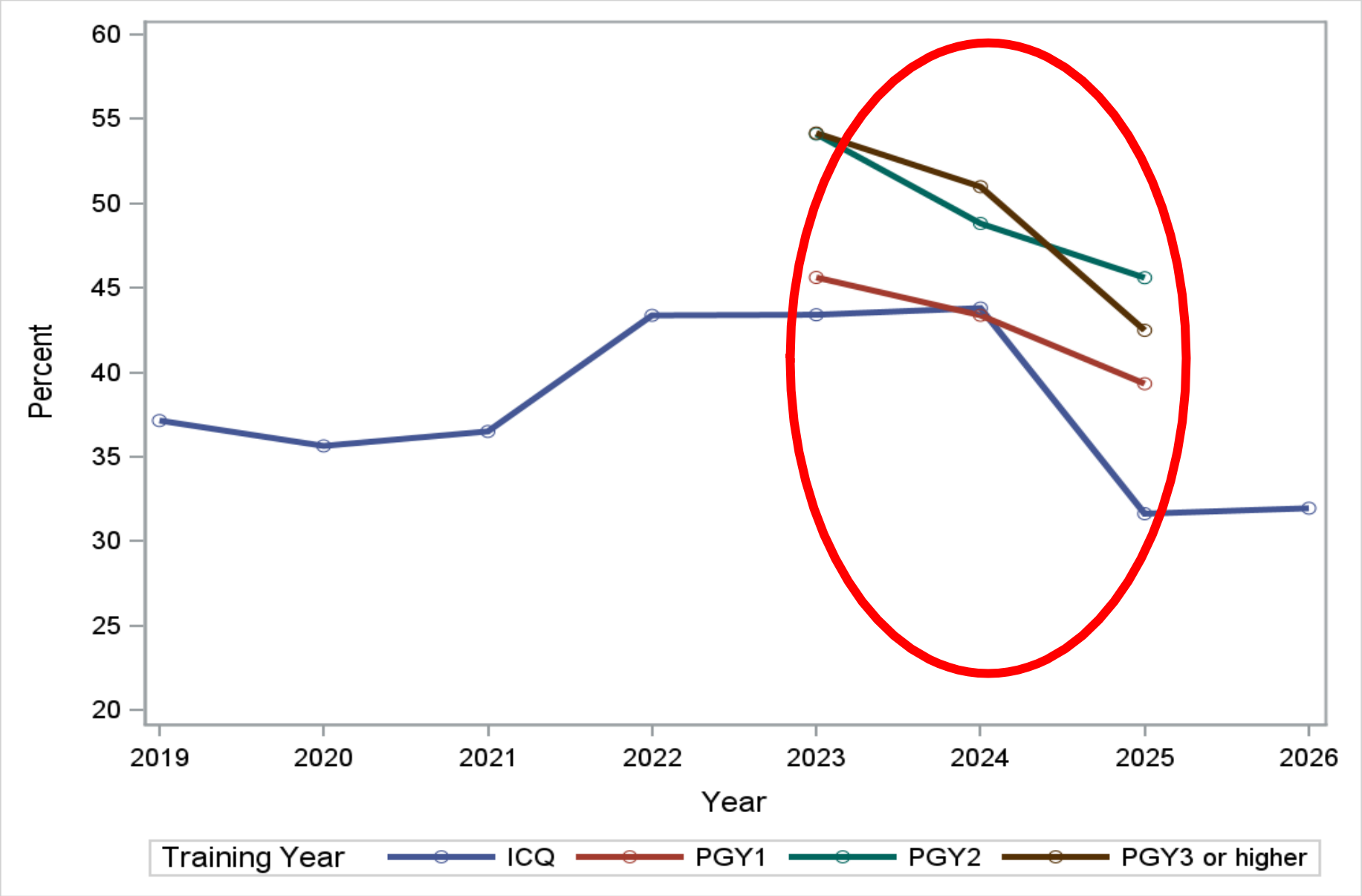
Too Much Pajama Time!

On average, how many hours per day do you spend outside of normal office hours documenting clinical care in your EHR?



AI tool used	PGY 1	PGY 2	PGY 3	Total
Physician Count:	5365	5024	4798	15187
Ambient documentation	12%	24%	31%	22%
Chart review	10%	10%	9%	10%
Medical reference	47%	39%	35%	41%
Assistance with administrative tasks	11%	12%	11%	11%
Clinical decision support	39%	31%	25%	32%
Scholarly activity	21%	18%	15%	18%
I don't use AI tools	27%	29%	31%	29%

Proportion with burnout symptoms decreasing!





Learnings and Next Steps...

- **Glass half full**--excellent progress at year 3 out of ? 8
- Transformation of residency practices with empanelment and increased continuity
- Dramatic increases in initial/ ongoing assessments and Individual Learning Plans
- Some RC requirements need modification...
- **Summer 2026: Pilot of Pre-ITE**
- **Minor Revision RC Guidelines:**
 - Curriculum, Practice Standards
 - Career Development Electives
 - Graduate Surveys in PEC
- **ACGME:** whither scholarship?, core/detail, CPR changes, ?shared data
- **ABFM:** with partners, evaluate faculty development, improve data systems, ? tracking during residency

**Comments,
Questions?**

